

Task 5 – Individual Case Safety Report (ICSR)

Pharmacovigilance Internship Submission | Mock Capstone Case

Case ID: VWL-ICSR-005

1. Case & Patient Information

Report Type	Initial report
Date Received	August 9, 2026
Source	Treating physician
Patient	Male, 56 years
Weight	78 kg
Medical History	Osteoarthritis of the right knee; controlled hypertension; no known history of gastrointestinal bleeding; no known drug allergies
Reporter	Treating physician, healthcare professional
Reporter Contact	Not provided

2. Suspected Drug

Drug	Diclofenac
Dose	50 mg
Route	Oral
Frequency	Twice daily
Indication	Osteoarthritis-related knee pain
Therapy Start	August 1, 2026
Therapy Stop	August 8, 2026
Action Taken	Drug discontinued

3. Concomitant Medicines

Amlodipine	5 mg once daily; oral; hypertension; used for 3 years
Paracetamol	500 mg as needed; oral; knee pain; occasional use for 6 months

4. Adverse Event

Reported Symptoms	Black, tarry stools, dizziness and general weakness
Date of Onset	August 8, 2026
Hospitalization	Admitted on August 8, 2026 for suspected gastrointestinal bleeding
Laboratory Findings	Hemoglobin 8.9 g/dL; stool examination positive for occult blood
Endoscopy	Gastric ulcer with active bleeding
Clinical Diagnosis	Acute upper gastrointestinal bleeding associated with a gastric ulcer

5. Management & Outcome

Management	Diclofenac discontinued immediately; intravenous proton-pump inhibitor therapy, fluid replacement and endoscopic treatment provided
Blood Transfusion	Not required
Outcome	Recovered; melena, dizziness and weakness resolved and hemoglobin stabilized
Discharge	August 12, 2026
Follow-up	August 20, 2026: clinically stable

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6. ICSR Validity Assessment

Identifiable Patient	Yes
Identifiable Reporter	Yes – treating physician
Suspected Medicinal Product	Yes – diclofenac
Adverse Event	Yes – upper gastrointestinal bleeding associated with gastric ulcer
Minimum Case Criteria	Met
ICSR Validity	Valid

7. ADR Identification

The patient developed an acute upper gastrointestinal bleed with an endoscopically confirmed gastric ulcer after seven days of diclofenac therapy. The event occurred after exposure to the suspected drug and improved following discontinuation and treatment. No newly introduced concomitant medicine was reported.

Suspected Drug	Diclofenac
Adverse Reaction	Gastric ulcer haemorrhage / acute upper gastrointestinal bleeding
Temporal Relationship	7 days from treatment initiation to onset
Alternative Causes	No obvious alternative drug-related trigger identified from the case

8. MedDRA Coding

Based on the documented clinical diagnosis of a gastric ulcer with active bleeding, the selected MedDRA Preferred Term is **Gastric ulcer haemorrhage** (PT code **10017826**). This is the most specific teaching code for the confirmed clinical event. In real case processing, the term should be checked against the organization's licensed MedDRA version and coding conventions.

Reported Clinical Event	Gastric ulcer with active bleeding / upper GI bleeding
MedDRA Preferred Term	Gastric ulcer haemorrhage
MedDRA Code	10017826
Coding Note	Melaena and dizziness are clinical features; the main coded event is the confirmed bleeding gastric ulcer.

9. Seriousness, Severity & Expectedness

Seriousness	Serious
Seriousness Criterion	Inpatient hospitalization
Severity	Severe
Expectedness	Expected for this mock case
Clinical Impact	Significant
Reason	The event required hospitalization and endoscopic treatment. The mock reference safety information states that gastrointestinal ulceration and bleeding are established adverse reactions to diclofenac.

10. Causality – Naranjo Algorithm

#	Question	Answer	Score
1	Previous conclusive reports on this reaction?	Yes	+1
2	Did the event appear after the suspected drug was administered?	Yes	+2
3	Did the reaction improve after discontinuation or a specific antagonist?	Yes	+1
4	Did the reaction reappear on readministration?	Do not know	0
5	Are there alternative causes that could have caused the reaction?	No	+2
6	Did the reaction reappear with a placebo?	Do not know	0
7	Was the drug detected at a known toxic concentration?	Do not know	0
8	Was the reaction more severe with dose increase or less severe with dose decrease?	Do not know	0
9	Did the patient have a similar reaction to the same/similar drug previously?	No	0
10	Was the event confirmed by objective evidence?	Yes	+1

Total Naranjo Score: 7 — Probable (5–8)

The score is supported by the temporal relationship, improvement after withdrawal, absence of an identified alternative cause, and objective clinical evidence from laboratory testing and endoscopy. Rechallenge was not performed.

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11. ICSR Narrative

A 56-year-old male weighing 78 kg with a history of osteoarthritis of the right knee and controlled hypertension was started on oral diclofenac 50 mg twice daily on August 1, 2026 for osteoarthritis-related knee pain. His concomitant medicines were amlodipine 5 mg once daily, used for three years, and occasional paracetamol 500 mg, used for six months.

On August 8, 2026, after seven days of diclofenac treatment, the patient developed black, tarry stools, dizziness and general weakness and was admitted to hospital. Laboratory assessment showed a hemoglobin level of 8.9 g/dL and a positive stool occult blood test. Upper gastrointestinal endoscopy demonstrated a gastric ulcer with active bleeding. The event was documented as acute upper gastrointestinal bleeding associated with a gastric ulcer.

Diclofenac was discontinued immediately. The patient received intravenous proton-pump inhibitor therapy, fluid replacement and endoscopic treatment. No blood transfusion was required. His condition improved, melena and dizziness resolved, and hemoglobin stabilized. He was discharged on August 12, 2026. At follow-up on August 20, 2026, he remained clinically stable.

Based on the available information, the event was classified as serious due to hospitalization and severe in intensity. For this mock case, the event was considered expected because gastrointestinal ulceration and bleeding are included in the stated reference safety information. The Naranjo score was 7, supporting a Probable causal relationship with diclofenac.

12. Final ICSR Assessment

ICSR Validity	Valid
Suspected Drug	Diclofenac
MedDRA PT	Gastric ulcer haemorrhage (10017826)
Seriousness	Serious – hospitalization
Severity	Severe
Expectedness	Expected
Causality	Probable – Naranjo score 7
Action Taken	Diclofenac discontinued
Outcome	Recovered

13. Learning Summary

This case demonstrates an end-to-end safety assessment: confirming ICSR validity, identifying the suspected adverse reaction, documenting the medicinal product and concomitant medicines, selecting an appropriate MedDRA Preferred Term, assessing seriousness/severity/expectedness, applying the Naranjo algorithm, and preparing a concise chronological ICSR narrative.

References used for learning

Naranjo Adverse Drug Reaction Probability Scale – NCBI LiverTox. MedDRA terminology verification – MedDRA terminology listings. The Naranjo scale interprets scores of 5–8 as Probable.